Healthcare Network Analytics – Executive Dashboard

Patient demand, billing and emergency activity across the network: what leadership needs to know.

Reporting period: May 2019 – May 2024

MEDICAL CONDITION: All conditions | INSURANCE PROVIDER: All providers | ADMISSION YEAR (COST TREND): All years | Reset filters

TOTAL ADMISSIONS	TOTAL BILLING	AVERAGE BILLING	AVERAGE PATIENT AGE
54,966	1.41B	25,595	51.5
Records across the network	Sum of all billing	Per admission	Range 13–89

EMERGENCY ADMISSIONS	ABNORMAL EMERGENCY CASES	HIGH-COST PATIENTS	CURRENT SELECTION
18,102	6,038	5,497	All conditions and providers
32.9% of all admissions	33.4% of emergency admissions	Top 10% by billing	

EXECUTIVE SUMMARY

Balanced demand
Six conditions each account for 16.5%–16.8% of patients; Arthritis (9,218) and Diabetes (9,216) lead.

Stable payer mix
Five insurers each cover about one in five patients; Cigna is largest at 20.3%.

Steady billing
Average billing is 25.6K per admission with no sustained upward trend over 61 months.

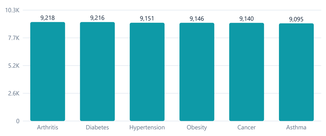
Age is not a cost driver
Patients aged 60+ average 25,564, in line with the overall 25,595.

Emergency pressure is constant
18,102 emergency admissions; monthly volumes stay between 1,452 and 1,573.

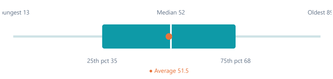
Review workload
6,038 emergency cases have abnormal results and 5,497 patients sit in the top billing decile.

KEY BUSINESS INSIGHTS

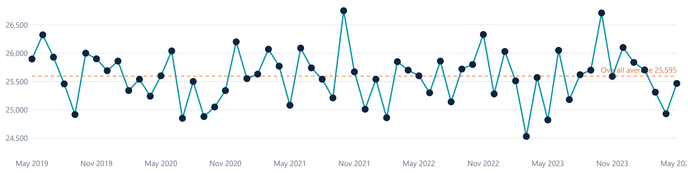
Arthritis is the most common condition, but demand is evenly spread
Arthritis (9,218) leads; Asthma is lowest (9,095), a gap of only 1.3%. Click a bar to filter.



Patients are mid-life on average, from teens to late 80s
Average age is 51.5; the middle half of patients are aged 35–68.



Average billing moves month to month without a sustained climb
Monthly average billing swings roughly between 24.5K and 26.8K around the overall 25.6K average; May 2024 (25,465) ended 1.7% below May 2019 (25,897).

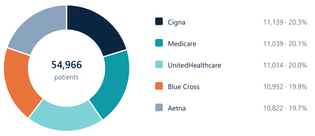


Monthly values other than the first and last five months are read from the analysis chart and are approximate.

Age does not drive cost
Patients aged 60+ average 25,564 per admission versus 25,595 overall, a difference of just 31 (0.1%).



Cigna covers the most patients, yet insurers are almost equal
Cigna (11,139) leads, Aetna is lowest (10,822); every provider holds 19.7%–20.3%. Click a slice to filter.



AD-HOC INSIGHTS

Emergency admissions are steady all year, with peaks in July and January
18,102 emergency admissions across the period; monthly totals range from 1,452 (February) to 1,573 (July), grouped by calendar month.

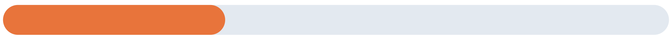


High-cost patients for finance review
Patients billed above the 90th percentile.

5,497

Above 90th percentile: 5,497 (10.0%)

One in three emergency patients has abnormal test results
6,038 of 18,102 emergency admissions (33.4%) are flagged for priority review.



Abnormal results: 6,038 (33.4%)

MANAGEMENT TAKEAWAYS

Demand is evenly spread across conditions
What is happening: Each of six conditions makes up about one-sixth of patients (9,095–9,218).

Why it matters: No single specialty dominates, so capacity is needed broadly rather than in one area.

Pay attention to: Arthritis and Diabetes, the two largest groups.

Emergency volume is a constant baseline
What is happening: 18,102 emergency admissions, 1,452–1,573 per calendar month.

Why it matters: Staffing can be planned on a stable level rather than sharp seasonal swings.

Pay attention to: July (1,573) and January (1,547) are the busiest months.

A third of emergency patients show abnormal results
What is happening: 6,038 emergency cases (33.4%) are flagged.

Why it matters: These cases carry the highest clinical priority and drive review workload.

Pay attention to: Capacity of the review process for flagged cases.

Costs are stable; high-cost cases are identifiable
What is happening: Average billing is steady near 25.6K, and 5,497 patients exceed the 90th percentile.

Why it matters: Finance can focus review on a defined 10% group instead of monitoring everyone.

Pay attention to: Spikes such as late 2021 and late 2023 in monthly averages.

Age does not explain billing
What is happening: 60+ patients average 25,564 vs 25,595 overall.

Why it matters: Age-based cost assumptions are not supported by this data.

Pay attention to: Other drivers behind high-cost cases.

Payer exposure is not concentrated
What is happening: Insurers hold 19.7%–20.3% each; Cigna 11,139 patients.

Why it matters: No single payer creates dependency risk.

Pay attention to: Cigna and Medicare, the two largest.